

Specialists and procedure breadth

Who we can attract, and what each of them can actually do here.

01 / WHAT SETS THE LIMIT

Anaesthesia and recovery, not specialty.

No specialty is admitted or excluded as a whole. What decides whether a given procedure can be done here is the anaesthesia it needs and the recovery it demands. Three tiers, and the facility either has the room or it does not.

Tier	Anaesthesia	Recovery	Room required	Available?
Tier 1	Topical or none	None to 15 min, ambulant	Treatment room, first floor, Playroom 22.4 sqm	Yes, today
Tier 2	Local infiltration	30 to 60 min, walks out	Treatment room, or the FUE suite for all-day work	Yes, today
Tier 3	Sedation or general	2 to 6 hours, monitored	Theatre with level trolley evacuation to an ambulance	Only with the new unit

The ground floor is Alameda's, so the theatre is a new unit on the grounds. That is the right answer anyway. A theatre needs positive-pressure ventilation with HEPA filtration at 15 to 25 air changes an hour, isolated power supply panels, and ceiling height for the ductwork and operating light. A residential room cannot give you any of the three without structural work, and we are holding to no wall removal. A purpose-built ground-level unit solves ventilation, power and evacuation together, at NGN 90M to 150M.

02 / SURGICAL AND PROCEDURAL SPECIALTIES

Twelve specialties, tier by tier.

Read across. Columns 2 and 3 are deliverable now. Column 4 requires the theatre.

Specialty	Tier 1 treatment room	Tier 2 local anaesthesia	Tier 3 theatre only
Plastic and aesthetic surgery	Injectables, fillers, peels, lasers, HIFU, cryolipolysis, PRP, thread lifts	Lesion and mole excision, earlobe repair, scar revision, upper-lid blepharoplasty, labiaplasty, small lipoma	Liposuction, abdominoplasty, breast augmentation and reduction, gynaecomastia, fat transfer, facelift, rhinoplasty
Hair restoration	PRP scalp, scalp micropigmentation, mesotherapy	FUE and FUT hair transplant, 6 to 10 hours, dedicated suite	Not required
General surgery	Wound care, dressings	Lipoma, sebaceous cyst, ingrowing toenail, lymph node biopsy, breast lump excision	Inguinal and umbilical hernia, haemorrhoidectomy, pilonidal sinus, adult circumcision
Urology	Bladder scan, uroflowmetry	Flexible cystoscopy, prostate biopsy under ultrasound, vasectomy	Hydrocele and varicocele repair, circumcision, TURP-lite selected cases
Ophthalmology	Intravitreal injections, YAG and SLT laser, retinal imaging	Chalazion, pterygium, minor lid lesions	Cataract phacoemulsification, full blepharoplasty, ptosis repair
ENT	Microsuction, nasal endoscopy, audiology	Nasal cautery, minor polypectomy	Septoplasty, turbinate reduction, grommets, tonsillectomy in selected patients
Orthopaedics and sports medicine	Joint and soft-tissue injections, PRP, ultrasound-guided injection	Ganglion excision, trigger finger release	Carpal tunnel release, knee arthroscopy, removal of metalwork
Obstetrics, gynaecology and fertility	Colposcopy, IUD insertion and removal, pessary, ultrasound	Endometrial biopsy, Bartholin cyst, LLETZ, hysteroscopy without sedation	Hysteroscopy and polypectomy under sedation, oocyte retrieval, dilatation and curettage

Dermatology	Cryotherapy, laser, phototherapy, patch testing, dermoscopy	Punch and excisional biopsy, curettage, skin cancer excision	Wide local excision with flap or graft
Vascular	Duplex scanning, sclerotherapy	Ambulatory phlebectomy, port and PICC insertion	Endovenous laser or radiofrequency ablation of varicose veins
Oral and maxillofacial, dental	Restorative, hygiene, whitening, smile design	Simple extractions, implant placement, bone grafting	Surgical third molars, multiple implants under sedation
Pain medicine	Trigger point and joint injections	Peripheral nerve blocks under ultrasound	Epidural and facet injections, radiofrequency ablation with image guidance

03 / CONSULTING AND DIAGNOSTIC SPECIALTIES

Twelve more that need no procedure room.

These fill the eight first-floor consulting rooms and drive the laboratory and imaging baskets. Several are not optional: a surgical campus cannot run without cardiology for pre-operative clearance, psychology for body-image and bariatric assessment, and physiotherapy for post-operative rehabilitation.

Specialty	What they do here, and why they belong
Cardiology	Echo, ECG, Holter, ambulatory BP, exercise tolerance testing. Also provides pre-operative clearance for every Tier 3 case on the campus
Endocrinology and metabolic	Diabetes, thyroid, obesity and GLP-1 programmes. Heaviest laboratory basket on the campus
Nephrology	Chronic kidney disease monitoring, hypertension, transplant follow-up
Respiratory	Spirometry, sleep screening, asthma and COPD review
Neurology	EEG, nerve conduction studies, headache and epilepsy clinics
Rheumatology	Inflammatory arthritis, biologics monitoring, joint injection in the treatment room
Gastroenterology	Consultation and follow-up. Endoscopy needs a dedicated decontamination room and is a separate future decision, not part of the theatre
Paediatrics	General paediatrics, growth and development, allergy. Feeds The Cloister family membership
Psychiatry and psychology	Long consultations that fill the evening bands. Also provides body-image and bariatric psychological clearance, which surgical aesthetics genuinely requires
Medical oncology and haematology	Consultation, surveillance and survivorship. Infusion if Medlyfe takes first-floor rooms later
Nutrition and dietetics	Pre and post-surgical optimisation, weight programmes
Physiotherapy	Post-operative rehabilitation and lymphatic drainage after liposuction

Twenty-four specialties in total, and the second twelve are what make the first twelve work. Recruit for the pathway, not for prestige. A surgical aesthetics practice that has to send patients out for a cardiology clearance, a psychological assessment and six weeks of lymphatic drainage is a worse practice, and every one of those referrals is revenue leaving the building.

04 / WHAT THE THEATRE ADDS

Including for Alameda.

Who benefits	How
Alameda Conversion Clinic	Cases that do not need tertiary care get done in Abuja instead of flying to Cairo. Alameda earns locally on the surgery, Medbury earns the theatre fee, and Cairo is reserved for complex and inpatient work where it genuinely adds value. This makes the conversion proposition stronger, not weaker
Aesthetic and plastic surgeons	Liposuction, abdominoplasty, augmentation and rhinoplasty become possible. Without a theatre the anchor can only sell injectables, devices and minor excision
Ophthalmology	Cataract surgery is the highest-volume day case in the world and it is a single theatre list. On its own it can fill two days a week
General surgery and urology	Hernia, haemorrhoids, hydrocele and circumcision are bread-and-butter day cases with reliable demand
Fertility	Oocyte retrieval needs sedation and a theatre. Without one, an IVF service cannot be complete
The plaza as a whole	A theatre is the single strongest recruitment argument available. A surgeon chooses an address where they can operate

05 / RECRUITMENT ORDER

Who to sign first, and why.

Specialty	Why in this order
1 Aesthetic or plastic surgeon	The anchor and the theme. Sets the campus identity and uses every tier

2	Hair restoration, FUE	Best yield per sqm in the building, one patient a day, no theatre needed. Its own dedicated suite
3	Dermatology	The clinical spine of an aesthetics theme, and it brings Tier 2 surgical volume of its own
4	Cardiology	Not optional. Pre-operative clearance for every Tier 3 case, and it carries the echo utilisation
5	Endocrinology and weight management	Fastest-growing adjacency in the GLP-1 era, and the heaviest laboratory basket
6	Ophthalmology	The theatre's most reliable list. Sign once the theatre is funded
7	Obstetrics, gynaecology and fertility	High willingness to pay, ultrasound intensive, and a theatre user
8	Psychology and physiotherapy	Complete the surgical pathway and stop referral leakage

06 / WHAT THIS FACILITY CANNOT DO

Stated plainly, so nobody is sold a promise.

Not possible here	Why
Anything requiring an overnight stay	There are no inpatient beds and there is no plan for any. Day case means home the same day
Major joint replacement, spinal, cardiac, neurosurgery	Needs inpatient beds, ICU backup and a blood bank
Anything needing intensive care backup	Patient selection must exclude it, which means firm ASA and BMI criteria
Full endoscopy service	Needs a dedicated scope decontamination room. A separate decision, and not solved by the theatre
Obstetric delivery	No labour or delivery capability, and none intended
Emergency and trauma	No accident and emergency function. Walk-in emergencies get stabilised and transferred
Radiotherapy, dialysis, chemotherapy day unit	Not in scope. Infusion only if Medlyfe takes first-floor rooms later

07 / ANAESTHETISTS ON ROSTER

What it unlocks, and what it obliges.

Putting anaesthetists on roster raises the clean procedure room from local infiltration to **conscious and moderate sedation, monitored anaesthesia care and regional blocks**. That unlocks longer FUE lists run comfortably, larger excisions, blepharoplasty with sedation, hysteroscopy, some endoscopy and paediatric imaging sedation, all without a theatre.

It does not relax the evacuation rule. It tightens it. A sedated patient cannot self-evacuate, so sedation raises the standard rather than lowering it. Sedation happens in the **ground-floor clean procedure room only**, where there is level trolley access to an ambulance. The first-floor treatment room stays local-only whoever is on the roster.

What a competent anaesthetist will not work without

Requirement	Note	NGN M
Capnography, end-tidal CO2	Standard of care for any sedation. Not optional and not negotiable	3 to 5
Anaesthetic machine or piped oxygen with backup	Plus scavenging if any volatile agent is used	8 to 15
Difficult airway trolley	Laryngoscopes, supraglottic airways, bougie, cricothyroidotomy kit	2 to 3
Defibrillator and resuscitation trolley	With reversal agents, flumazenil and naloxone	2 to 4
Monitored recovery bay	Pulse oximetry, NIBP, ECG, and a nurse trained in post-anaesthetic care to formal discharge criteria	2 to 3
Additional capital over the base room		17 to 30

That takes the clean procedure room from NGN 25M to 35M up to **NGN 45M to 60M**. Two further obligations carry no capital but are absolute: the **two-person rule**, meaning whoever administers sedation cannot also perform the procedure, so every sedation case needs proceduralist, anaesthetist and nurse; and a **written transfer agreement** with a nearby hospital with theatre and ICU capability, which is both a licensing expectation and an insurance condition.

Roster shape, and who carries the cost

Option	Assessment
Fixed sessions, recommended	Two anaesthetist days a week with all sedation cases scheduled into them. Makes the roster affordable, gives proceduralists a predictable slot, and means recovery staffing is only needed on those days
On-call roster	Available with notice, booked per case. Cheapest, least reliable, and hard to staff recovery around
Retainer plus case fee	A small availability retainer plus a per-case fee. Worth considering once volume is proven

Do not carry the anaesthetist on the facility payroll. At NGN 200,000 a session against 138 sold sessions, absorbing the cost would leave the sedation-capable room earning *less* than the local-only room. The right structure is that The Lyfe Place provides a compliant room and rosters the anaesthetist, and the fee is **billed through to the operating clinician with a coordination margin**. Room at NGN 480,000 a session plus a roughly NGN 40,000 coordination margin gives about **NGN 72M a year against NGN 44M** for the local-only room, and the facility carries no sessional risk.

One item to confirm rather than assume: FCT health facility registration will need to reflect sedation capability, and there may be specific conditions attached. Worth establishing before the equipment is ordered.

08 / WHAT THE THEATRE OBLIGES

Governance, before the first list.

A theatre is not a room, it is a clinical governance system. These are conditions of opening, not refinements.

Obligation	What it means
Anaesthetist cover and a pre-assessment clinic	Named consultant anaesthetist, and every Tier 3 patient assessed before the list
Written patient selection criteria	ASA I to II, BMI ceiling, a responsible escort home, no untreated sleep apnoea, distance from the facility
Resuscitation capability	Full trolley, difficult airway equipment, defibrillator, and an ALS-trained team on site whenever the theatre is in use
A written transfer agreement	With a named receiving hospital, signed before the first case, not after the first problem
Discharge criteria and follow-up	Written, auditable, with a 24-hour contact number
Sterile services	Either a decontamination room and autoclave on site, or an outsourced CSSD contract with tracked instrument sets
Theatre register, WHO checklist, audit	Logbook, safety checklist every case, and a morbidity review that actually meets
Regulatory	FCT health facility registration extended to surgical services, and MDCN registration for every operating clinician including visiting Egyptian specialists

The honest sequence. Open on Tiers 1 and 2, which need nothing beyond the treatment room and the FUE suite you already have, and run the theatre-requiring specialties through a partner hospital in the meantime. Fund the theatre unit when there is a booked list to justify it, which on ophthalmology and general surgery alone should be inside eighteen months. Building it first and looking for surgeons afterwards is the expensive way round.

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Procedure lists follow standard day case and office-based practice and are indicative, not a scope of practice. Every case mix must be confirmed by a clinical governance review and against each practitioner's own credentials and indemnity. Tier boundaries on sedation are conservative by design. Not medical advice. Companions: the engagement note, the space allocation drawing and the rate assumptions.